

CLOXACAP CAPSULE

VICLOxx-0 (MY)

DESCRIPTION

Cloxacap 250 mg Capsule

Orange opaque and grey opaque capsule with "HD" printed on one end and "CX 250" on the other end of the capsule.

Cloxacap 500 mg Capsule

Orange opaque and black opaque capsule with "HD" printed on one end of the capsule and "CX500" on the other end.

COMPOSITION

Cloxacap 250 mg Capsule

Cloxacillin Sodium equivalent to Cloxacillin 250 mg/capsule.

Cloxacap 500 mg Capsule

Cloxacillin Sodium equivalent to Cloxacillin 500 mg/capsule.

PHARMACODYNAMICS

Cloxacillin is an isoxazoly penicillin which inhibit bacterial cell wall synthesis. Probably by acylation of membrane-bound transpeptidase enzymes. This prevents cross-linkage of peptidoglycan rigidity. Also, cell division and growth are inhibited and lysis of susceptible bacteria frequently occurs. Rapidly dividing bacteria are most susceptible to penicillins.

Cloxacillins markedly resistant to cleavage by penicillinase. The steric effects of the 3-O chlorophenyl and 5-methyl groups of the isoxazoly ring prevent the binding of this penicillin to the penicillinase receptor and thus protect the lactam ring from degradation. It is relatively stable in an acid medium and adequately absorbed after oral administration.

PHARMACOKINETICS

Absorption: Cloxacillin is acid stable and variably absorbed after oral administration; after an oral dose of 2mg of the sodium salt, 30 to 50% is absorbed.

Blood Concentration: After an oral dose of 500mg of the sodium salt, a peak plasma concentration of 5 to 12 µg per ml is attained within 30 to 60 minutes; effective concentrations are maintained for 4 to 6 hours after a single dose; After oral dose of 500mg given to children every 6 hours for 2 days, serum concentrations of about 7µg/ml are attained 2 hours after the last dose.

Half-life: Plasma half-life, 30 minutes to 1 hour, which is increased in renal function impairment.

Distribution: Well distributed throughout the tissue and does not enter the cerebrospinal fluid; concentrations in synovial fluid reach about half that in the serum during multiple dosing.

Protein binding: 80 to 95% bound to plasma protein

INDICATIONS

For treatment of:

- Infections caused by penicillinase-producing staphylococci
- Respiratory tract infections
- Skin and soft-tissue infections
- Disseminated infections caused by pneumococci, group A beta-hemolytic streptococci and penicillin G-resistant and penicillin G-sensitive staphylococci

CONTRAINDICATIONS

Avoid in patients known to be hypersensitive to penicillins.

WARNINGS AND PRECAUTIONS

Risk-benefit should be considered when the following medical problems exist:

- General history of allergy such as asthma, eczema, hay fever and hives.
- History of bleeding disorders.
- History of gastrointestinal disease especially ulcerative colitis, regional enteritis or antibiotic-associated colitis.

- Infectious mononucleosis.
- Caution in patients with known history of allergy and in severe renal function impairment.
- Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients receiving therapy with beta-lactams. Before initiating therapy with Cloxacap, careful inquiry should be made concerning previous hypersensitivity reactions to penicillins, cephalosporins, carbapenems or other beta-lactam agents. If an allergic reaction occurs, Cloxacap must be discontinued immediately and appropriate alternative therapy instituted.

SIDE/ADVERSE EFFECTS

- Hematologic reactions, specifically neutropenia (sore throat and fever) especially with high doses and platelet dysfunction (unusual bleeding or bruising).
- Pseudomembranous colitis (fever, rash, possible decreased urine output, proteinuria, hematuria).
- The most common adverse effect is a hypersensitivity reaction such as diarrhoea, nausea and vomiting.

PREGNANCY AND LACTATION

Cloxacillin is excreted in breast milk. Although no significant problems have been reported, the risk benefit must be considered since its use by nursing mothers may lead to sensitization of infants. Problems in humans have not been documented. However the risk-benefit must be considered.

OVERDOSAGE

Clinical features:

Anorexia, nausea, vomiting, abdominal discomfort and diarrhoea.

Treat overdosage by emesis or gastric lavage, if appropriate; and treat gastrointestinal distress, if any, by giving milk every 3 hours alternating with bismuth subcarbonate 5 g every 3 hours.

DRUG INTERACTIONS

- Aspirin, sulphamethoxypyridazine and sulphathiazole inhibits the serum binding of cloxacillin *in vitro* and *in vivo*.
- Incompatible with aminoglycoside, tetracyclines and other antimicrobial agents including erythromycin and polymyxin B-sulphate.

DOSAGE AND ADMINISTRATION

Cloxacillin should not be used orally for the initial treatment of severe life-threatening infections, but may be used as a follow-up therapy after parenteral penicillinase-resistant penicillin therapy.

Adult dose

Oral, 250 to 500 mg every six hours; or as directed.

Paediatric dose

Infants and children up to 20 kg of body weight:

Oral 12.5 to 25 mg per kg of body weight every six hours; or as directed.

Children 20 kg of body weight and over - See adult dose.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage: Store below 30°C. Protect from light and moisture.

Presentation/Packing:

Capsule 250 mg blister of 10 x 10's.

Capsule 500 mg blister of 10 x 10's

(Not all presentations are available locally.)

Product Registration Holder/Manufactured by: HOVID Berhad
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Revised date: August 2024